

CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: JonesPeriod: May 1 to June 1, 2026Report Date: June 2, 2026Coverage: 704/745h (94%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Patient clinical status over 32 days from May 01 to Jun 01

HR Breathing



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: GREEN

Vital	Avg	Min	Max
Heart Rate	55 bpm	39 bpm	74 bpm
Breathing	17 brpm	12 brpm	27 brpm

Last 24 hours: no episodic events, vitals within normal range.

Episodic Events (last 24h)

No episodic events in the last 24 hours.

9 episodic events Detected, spanning 12h total. Conditions: Low Heart Rate. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
9	12h	Low Heart Rate	<1	94%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 08	8 PM to 10 PM	2h	1	Low Heart Rate	43	40	49	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
May 11	3 AM to 4 AM	4h	3	Low Heart Rate	43	39	49	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
May 19	4 PM to 5 PM	5h	4	Low Heart Rate	44	41	51	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
May 25	12 AM to 1 AM	1h	1	Low Heart Rate	44	39	48	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

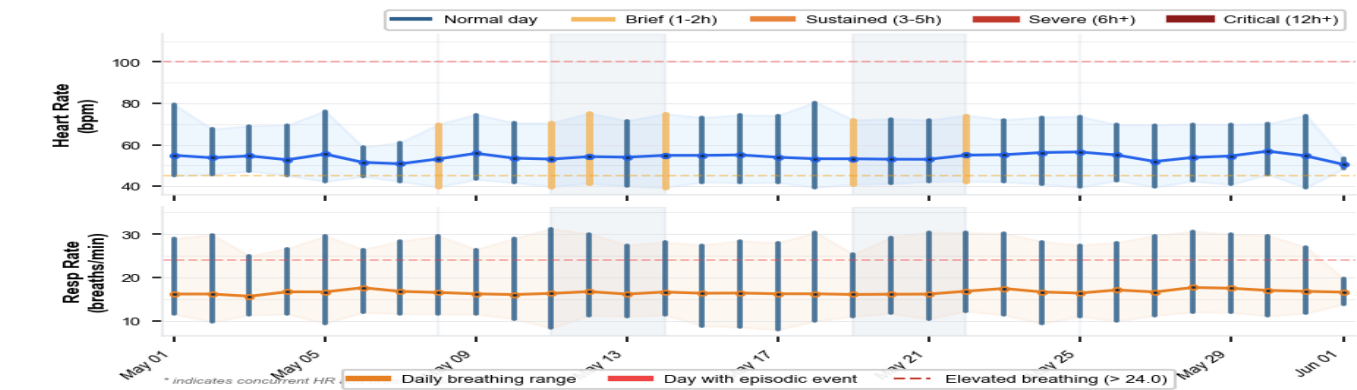
Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

Suggested Clinical Review Actions

- Low Heart Rate (avg 44 bpm, min 41 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 4 unstable phases with 16 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.
- Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

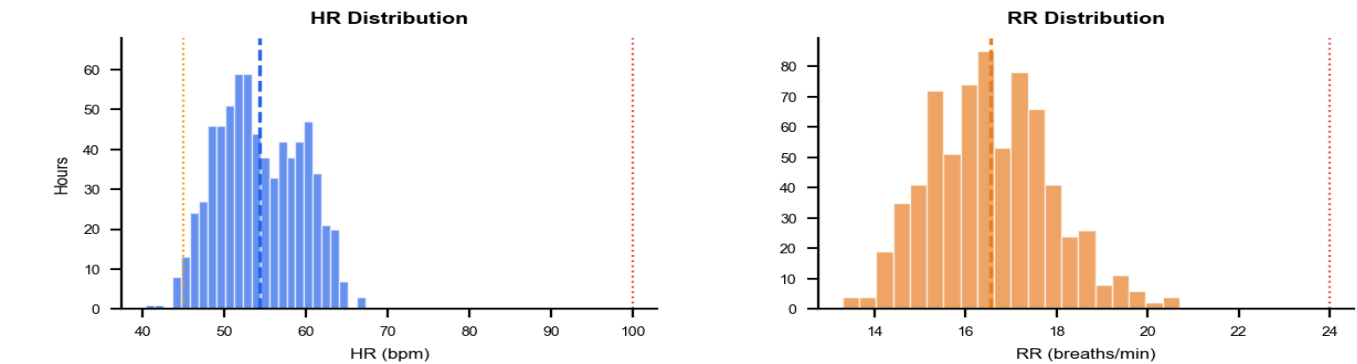
Clinical Pattern Observations:

- 1. Clustered pattern: Episodic events on 6 of 32 days (18%).
- 2. Nocturnal pattern: 4 of 6 eligible episodes during evening or night hours.

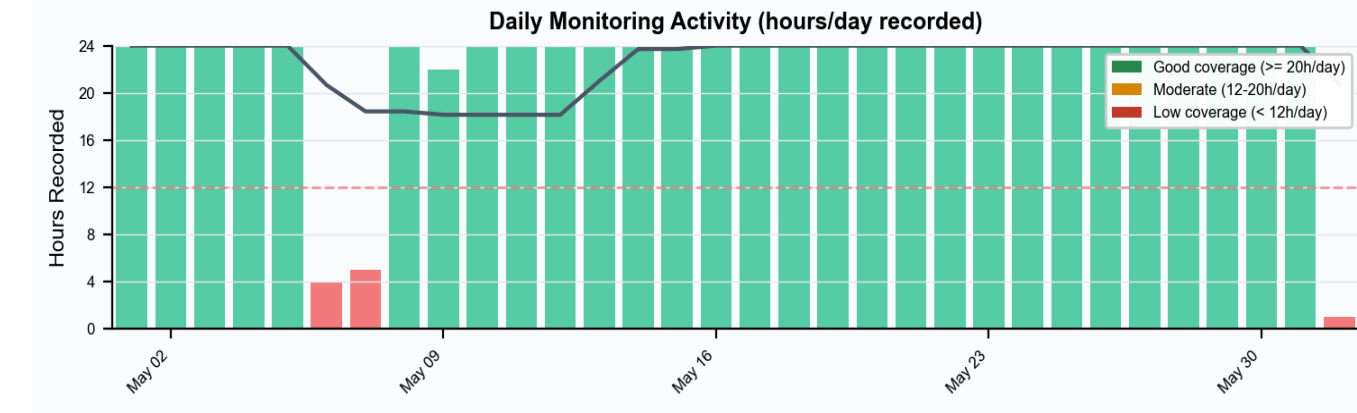


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.